FDA 510(k) fees and development-budget assumptions

Historical snapshot — October 10, 2026. Older app-status statements and commercial alternatives reflect this assessment date. See current implementation and the selected integrated device, app, and model research MVP. The existing-products/refill service is an alternative, not the selected MVP. Original evidence limits remain.

Product: One-The-Go Detection, United States consumer self-assessment.
Prepared: October 10, 2026, America/Chicago.
Status: Published FDA fees verified; other dollar amounts below are explicitly assumed planning allocations, not vendor quotes or a validated project estimate. This document does not change the intended use or close a release gate.
PDF: 510(k) cost assessment.

What would it cost?

The current FDA 510(k) filing fee is $28,653, or $7,163 for an FDA-qualified small business. That buys submission review, not the assay development, tests, quality records or clinical evidence needed to support clearance. Paying the fee does not guarantee clearance. [C01]

The repository is a specification and synthetic starter, with no validated assay, production device or trained impairment model. The actual total cannot yet be priced reliably. For a narrower custom OTC saliva THC screen, the example below allocates **$500,000 to evidence, quality and submission work**, before FDA fees and core product development. It is a transparent budgeting scenario to replace with quotes; it is not a statement that this product will cost $500,000 or that FDA requires that amount.

For the full saliva-plus-physiology/task impairment product, eligibility for a 510(k) has not been established. Do not treat this THC-screen scenario as the budget for developing and validating an impairment score. An alternative such as De Novo could have a different submission fee and substantially different evidence work.

Verified FDA fees: fiscal year 2027

FDA's published schedule applies **October 1, 2026 through September 30, 2027**. Its page states content current as of October 1, 2026; retrieved during this assessment. Recheck the applicable fiscal year before paying or submitting. [C01]

Fee	Standard	Qualified small business	Meaning
510(k) submission	$28,653	$7,163	Traditional, Abbreviated and Special 510(k)s are subject to the user fee.
Annual establishment registration	$13,785	$13,785 unless a separate waiver is granted	Separate recurring fee when the establishment's role requires registration/payment; not part of the 510(k) filing fee.
De Novo classification request	$191,020	$47,755	Alternative-pathway reference only; De Novo has not been determined necessary for this product.

The small-business 510(k) rate saves **$21,490 per submission** compared with the standard rate. It does not reduce the cost of laboratories, studies, consultants or engineering.

Small-business qualification

FDA must approve a Small Business Request for the relevant fiscal year. The reduced application-fee threshold is **$100 million or less in gross receipts or sales, including affiliates, for the most recent tax year**. Required tax documentation matters: being a new or small startup does not automatically establish eligibility. Obtain the applicable determination before relying on the reduced submission fee. [C02]

The first-premarket-application waiver described in the small-business program **does not apply to 510(k)s**. The annual registration fee has no routine small-business discount. A separate hardship waiver has additional conditions, including receipts/sales of $1 million or less with affiliates, financial hardship and evidence of prior paid registration; it excludes initial registration. Do not assume that waiver when budgeting a new establishment. [C01, C02]

The fee table notes that submissions made to FDA on behalf of an FDA-accredited third-party reviewer have no FDA 510(k) user fee. This is not free review: eligibility and the reviewer's charges require separate assessment. It has not been established as an option for this product. [C01]

Illustrative narrower-screen budget

Assumed scope: a custom OTC oral-fluid THC exposure screen, with a suitable pathway still to be confirmed. The core assay/reader design is assumed available before the work priced here. The result is exposure screening, not impairment, sobriety, safe-to-drive or clearance-time prediction.

The allocations below are assumptions selected to make the budget discussable. They are not sourced market averages, contracted prices, FDA-prescribed study sizes or a completeness determination. Some activities overlap; quotes should consolidate them to avoid double counting.

Workstream	Assumed allocation	Example scope to quote
Regulatory strategy and submission preparation	$50,000	Intended use, predicate comparison, submission assembly and responses.
Quality, design and risk records	$50,000	Applicable quality processes, traceability and risk-control evidence.
Assay, collector and reader analytical validation	$100,000	Reference comparisons, interference, repeatability and specimen/reader variation.
Intended-user, self-use and claim-supporting evidence	$125,000	Consumer-use performance and usability appropriate to the final screen claim.
Software, cybersecurity and hardware verification	$50,000	Relevant verification for the finished configuration and connectivity.
Manufacturing, lot and stability bridge	$25,000	Evidence connecting qualified design to production lots and shelf-life conditions.
Contingency	$100,000	Repeats, evidence gaps and review questions; not guaranteed sufficient.
Assumed program total before FDA fees	**$500,000**	**A scenario, not the actual cost of the product.**

Example initial-year arithmetic

This example assumes one 510(k), one establishment that must pay an annual registration fee, the FY 2027 schedule, and no registration waiver.

Scenario	Program assumption	Submission fee	One annual registration	Example total
FDA-qualified small business	$500,000	$7,163	$13,785	**$520,948**
Standard fee	$500,000	$28,653	$13,785	**$542,438**

Registration timing and liability depend on the actual establishment and activity; it is not a universal prerequisite charge for every applicant or for merely discussing a submission. Multiple establishments, additional submissions or later fiscal years change the arithmetic. [C03]

Costs excluded from the example

- Core chemistry invention, assay feasibility, hardware/app development and internal employee salaries.
- Training and independently validating the cannabis-impairment model, including its separate endpoint and study program.
- Production scale-up, tooling beyond the limited evidence bridge, inventory, marketing and distribution.
- Continuing annual registration, quality-system operation, calibration/service, complaints, postmarket work and later device changes.

A production-ready OEM system with usable evidence could reduce new work. A new collector, quantitative assay, automatic optical reader, self-use workflow or expanded claim could add work. No supplier prices or study protocols have been obtained for this project.

Practical next step

Obtain a scoped intended-use/predicate review before commissioning a full 510(k) package. Then request separate written quotes from the assay/reference laboratory, intended-user study provider, verification providers and regulatory/quality specialist. Specify existing evidence, deliverables, review-response coverage, repeat-test costs, subcontractors and exclusions. Replace the scenario's allocations with those quotes and an approved evidence plan.

The current full impairment claim needs its own route and evidence assessment. Narrowing the first release to exposure screening is a substantive product change; calling an impairment output wellness or research does not provide the same cost reduction.

Sources and evidence status

Sources accessed October 10, 2026, America/Chicago. Live FDA retrieval succeeded for this cost assessment; the earlier classification PDF records its own earlier access limitation. Fee verification here does not independently verify all classification assertions in the older reports.

ID	Primary source	What it supports
C01	FDA: Medical Device User Fee Amendments (MDUFA): Fees	FY 2027 filing/registration amounts, applicable dates and third-party submission note.
C02	FDA: Reduced or Waived Medical Device User Fees—Small Business Determination Program	Revenue/affiliate thresholds, determination process, tax documentation and waiver limitations.
C03	FDA: Who Must Register, List and Pay the Fee	Activity-specific establishment obligations; domestic distribution differs from manufacturing/importing.

Repository context: PRD, FDA regulatory assessment, faster market-entry strategy, and classification-options PDF. None provides vendor quotes or an established 510(k) route for the complete impairment function.